HCP Perception Survey: Cosentex in plaque psoriasis (EU, 2022)

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Executive Summary

This survey of 350 dermatologists across Germany, France, Italy, Spain, United Kingdom examined how clinicians perceive Cosentex (secukimab) for plaque psoriasis. Fieldwork took place in January 2022.

Overall HCP sentiment index for Cosentex in Germany: 0.70 (scale 0 to 1, assessed 2022). Respondents viewed the brand strongly positively.

Methodology

Respondents were dermatologists who treat at least 13 patients per month with plaque psoriasis. The sample size was n=350. Interviews lasted 35 minutes and were conducted online.

Results are weighted to national prescribing volumes. Statistical significance was assessed at the 95% confidence level.

Dermatologists emphasised that patient-reported outcomes and quality of life are increasingly important when choosing between IL-17A inhibitor options.

The unmet need in plaque psoriasis persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

Respondents expect real-world persistence on Cosentex to be lower than in trials, mainly because of discontinuation after early adverse events and the complexity of managing upper respiratory infections.

Regional variation within Spain is notable: large centres report faster uptake and broader use than smaller regional hospitals.

Subgroup analyses suggest that benefit with Cosentex is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Registry data show that outcomes in plaque psoriasis depend heavily on early diagnosis, access to specialist care and adherence to maintenance therapy.

Treatment burden, including monitoring visits and discontinuation after early adverse events, was described as a key factor in long-term adherence and persistence.

Dose modification guidance for Cosentex is generally followed, but dermatologists said that practical monitoring of injection-site reactions depends on local capacity and patient travel distance.

Peer-to-peer education and congress data were cited as the most trusted sources when dermatologists evaluate new evidence on IL-17A inhibitor agents.

Supply and logistics are not a concern for Cosentex, though cold-chain handling and nurse-led administration add workload in busy clinics.

Key Findings

Respondents highlighted the mechanism of action of Cosentex (neutralises interleukin-17a) as a differentiator in plaque psoriasis, although long-term safety data are still limited.

Real-world evidence and patient-reported outcomes were requested by 54% of respondents before expanding use beyond first-line therapy.

67% of respondents considered the efficacy of Cosentex clinically meaningful versus the current standard of care, with the strongest support in second-line use.

Prescribing intent for Cosentex among dermatologists was cautious, with 50% planning to increase use in the next 12 months.

Adherence concerns centred on monitoring requirements, particularly in patients with comorbidities and polypharmacy.

Respondents expect real-world persistence on Cosentex to be lower than in trials, mainly because of monitoring requirements and the complexity of managing nasopharyngitis.

Registry data show that outcomes in plaque psoriasis depend heavily on early diagnosis, access to specialist care and adherence to maintenance therapy.

Clinical guidelines in Germany recommend first-line use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

Treatment burden, including monitoring visits and monitoring requirements, was described as a key factor in long-term adherence and persistence.

Cost-effectiveness submissions typically model first-line use, and payers question the durability of benefit beyond the trial follow-up period.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Safety monitoring focused on injection-site reactions and nasopharyngitis, with dose interruption or discontinuation used for severe adverse events.

Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

Peer-to-peer education and congress data were cited as the most trusted sources when dermatologists evaluate new evidence on IL-17A inhibitor agents.

Medical education priorities include recognising injection-site reactions early, communicating realistic expectations, and coordinating with primary care.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Real-world evidence from registries suggests that outcomes in routine practice are broadly consistent with phase 3 trial results, although persistence and discontinuation rates are higher outside clinical trials.

Tolerability and Safety Perceptions

Tolerability was assessed against conjunctivitis and nasopharyngitis. Dermatologists reported managing these events with dose adjustment and supportive care.

Safety perception differed by experience: respondents with more than 13 treated patients rated Cosentex more favourably.

Guideline committees are expected to review the evidence base for plaque psoriasis again next year, which could change eligibility criteria and referral patterns.

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Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Medical education priorities include recognising upper respiratory infections early, communicating realistic expectations, and coordinating with primary care.

Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Respondents expect real-world persistence on Cosentex to be lower than in trials, mainly because of discontinuation after early adverse events and the complexity of managing nasopharyngitis.

Regional variation within Spain is notable: large centres report faster uptake and broader use than smaller regional hospitals.

Biosimilar and generic competition in adjacent classes is shaping price expectations, even where Cosentex has no direct substitute.

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Sequencing after progression remains an open question; dermatologists in Spain reported using a different mechanism of action after Cosentex fails.

Combination therapy and sequencing strategies remain debated; respondents noted limited head-to-head evidence between Cosentex and competing agents.

Conclusions

Cosentex is perceived strongly positively in Germany. Priorities for the brand team are monitoring requirements and evidence generation for plaque psoriasis.

Subgroup analyses suggest that benefit with Cosentex is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

Supply and logistics are not a concern for Cosentex, though cold-chain handling and nurse-led administration add workload in busy clinics.

Peer-to-peer education and congress data were cited as the most trusted sources when dermatologists evaluate new evidence on IL-17A inhibitor agents.

Cost-effectiveness submissions typically model refractory disease use, and payers question the durability of benefit beyond the trial follow-up period.

Real-world evidence from registries suggests that outcomes in routine practice are broadly consistent with phase 3 trial results, although persistence and discontinuation rates are higher outside clinical trials.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Respondents expect real-world persistence on Cosentex to be lower than in trials, mainly because of treatment burden over long durations and the complexity of managing upper respiratory infections.

Dose modification guidance for Cosentex is generally followed, but dermatologists said that practical monitoring of conjunctivitis depends on local capacity and patient travel distance.

Clinical guidelines in France recommend refractory disease use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

Registry data show that outcomes in plaque psoriasis depend heavily on early diagnosis, access to specialist care and adherence to maintenance therapy.

Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

The unmet need in plaque psoriasis persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

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Segment	Respondents	Sentiment index
High-volume prescribers	140	0.74
Low-volume prescribers	210	0.67

Synthetic document generated for the Watson prototype. All brand names and figures are fictional.